



A Multi-state Modeling Method to predict 10-year Disease trajectories between Coronary Heart Disease and Heart Failure

Mia Zhou¹, Albert Kang¹, Wayne Rosamond², Michelle Meyer^{2,3}, Ming Ding^{2,3}

¹Department of Biostatistics, ²Department of Epidemiology, ³Department of Emergency Medicine, University of North Carolina at Chapel Hill

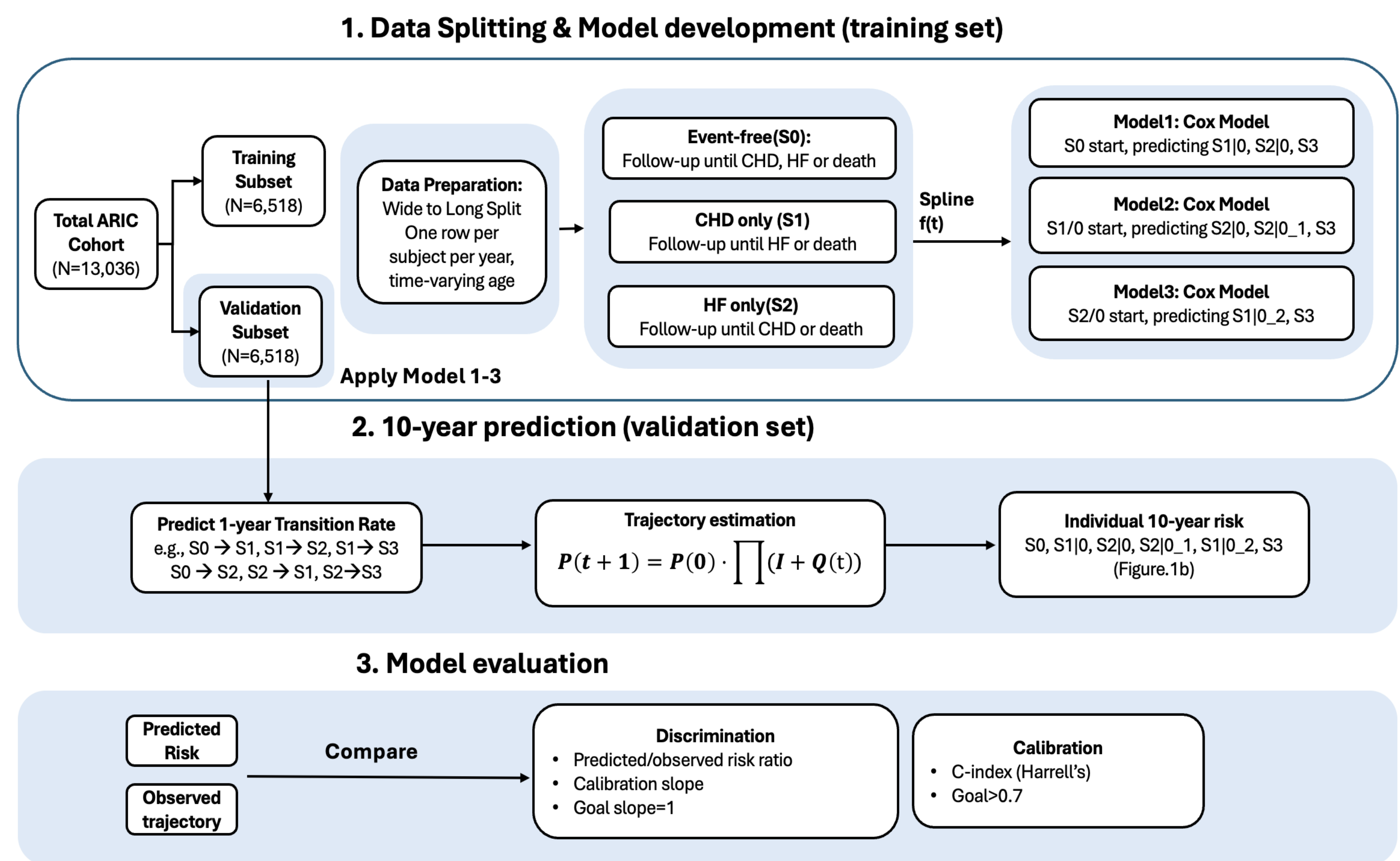
Background

Traditional cardiovascular risk models (e.g., PREVENT) only predict the **first event**, such as coronary heart disease (CHD) or heart failure (HF). However, patients often experience **multiple sequential cardiovascular events**, and these disease pathways have different prognoses and treatment strategies. Ignoring disease trajectories may underestimate disease complexity and limit personalized risk prediction.

Objective

To develop a multi-state model predicting an individual's 10-year risk of following each of six possible health trajectories after coronary heart disease (CHD) and heart failure (HF).

Methodology



Formula for step 1 models:

Model 1 starts from S_0 : $h_{(S_m=1|S_0=1)}(\tau, t, V) = h_{0m}(\tau) \exp(f(t) + V)$, $m = 1, 2, 3$.

Model 2 starts from S_1 : $h_{(S_m=1|S_1=1, S_0=1)}(\tau, t, V) = h_{1m}(\tau) \exp(f(t) + V)$, $m = 2, 3$.

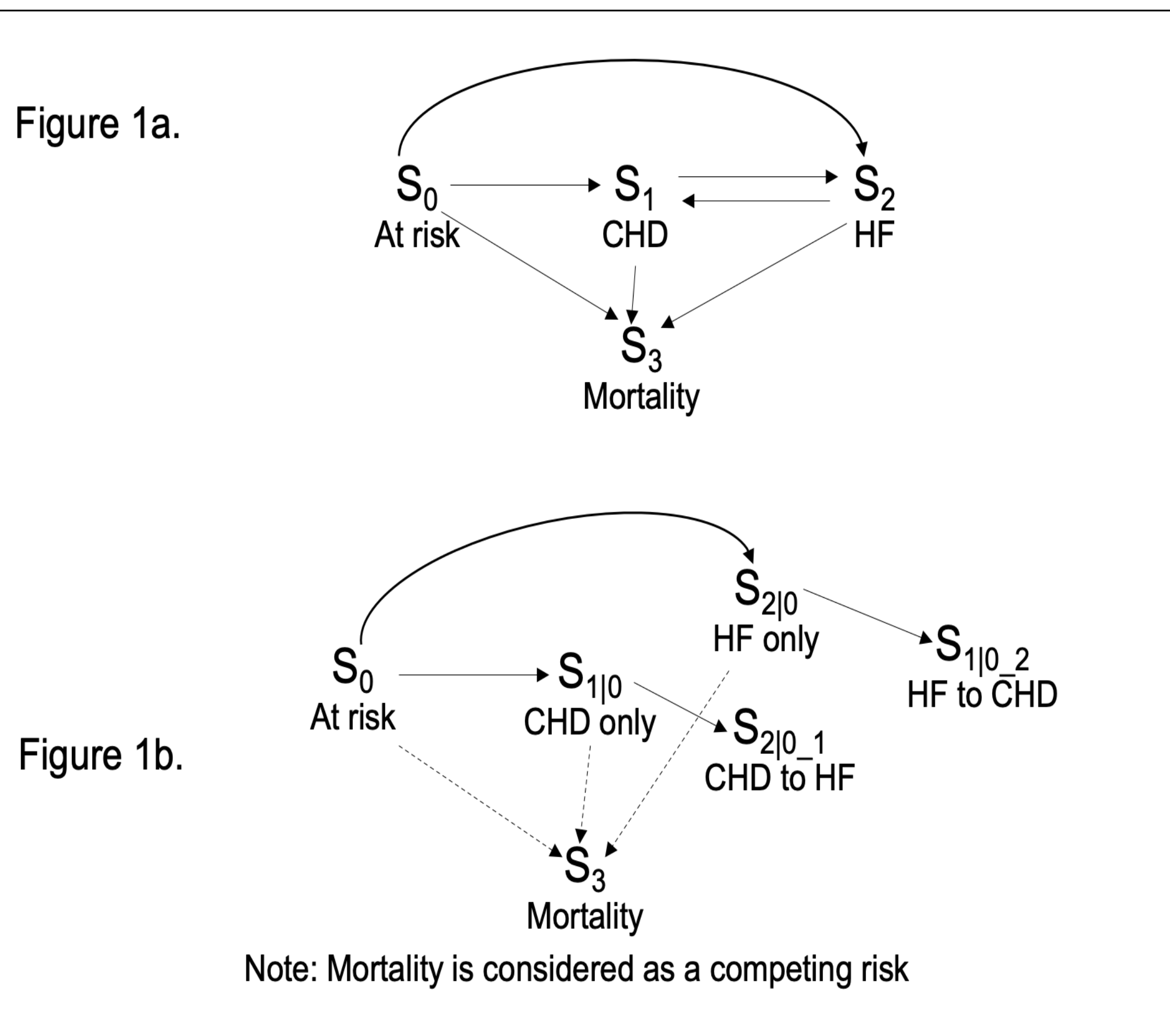
Model 3 starts from S_2 : $h_{(S_m=1|S_2=1, S_0=1)}(\tau, t, I_{0,1}, V) = h_{2m}(\tau) \exp(f(t) + V)$, $m = 1, 3$.

where $f(t)$ is a function of age, τ is time to event, $h_{0m}(\tau)$ is the baseline hazard of developing state m . V denotes the baseline predictors: sex, race, diabetes, smoking, blood pressure, lipids, eGFR, medication use.

References

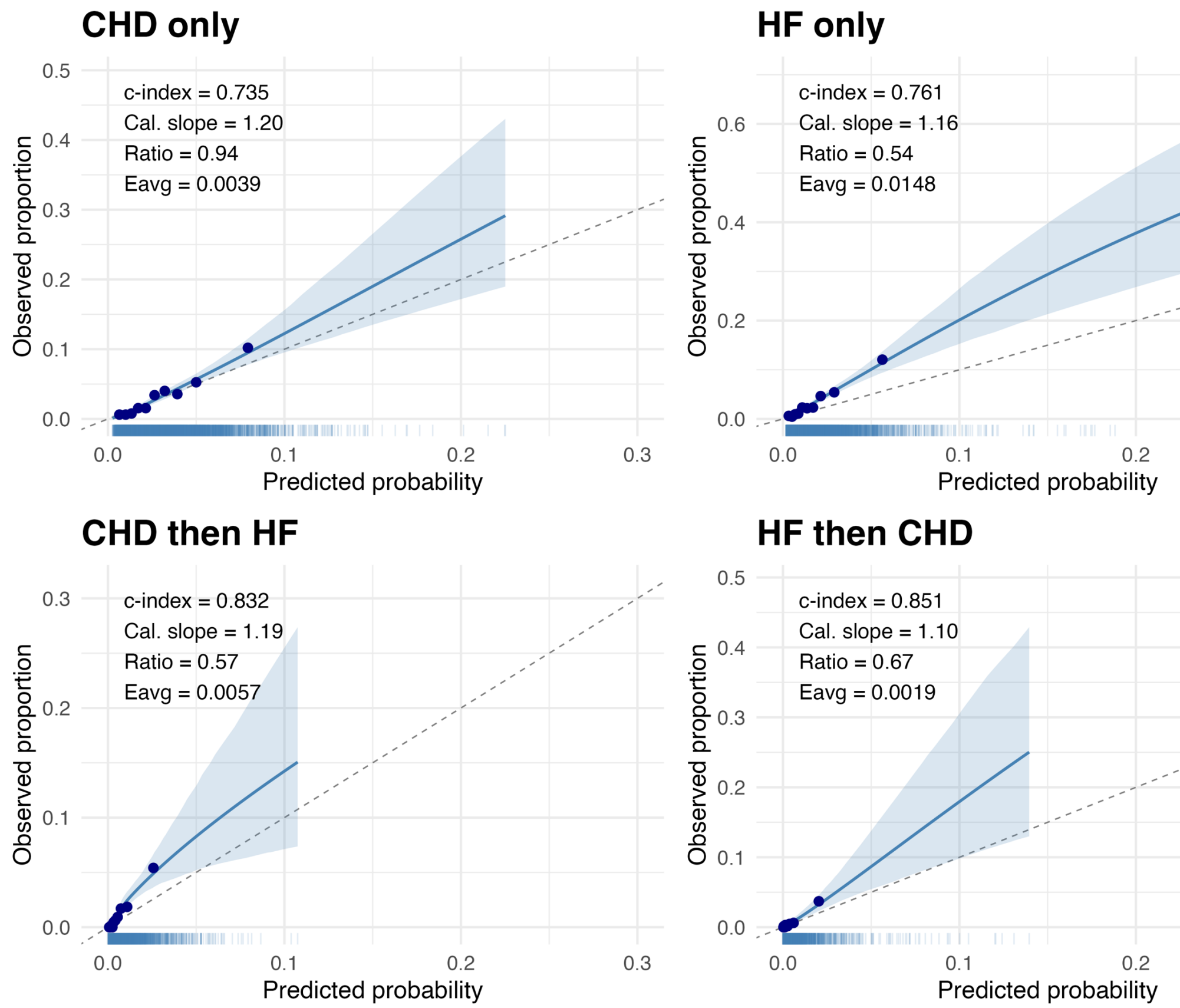
- Ding M, Chen H, Lin FC. A discrete-time split-state framework for multi-state modeling with application to describing the course of heart disease. BMC Medical Research Methodology. 2025;25(1):54.
- Ding M, Lin FC, Meyer ML. Summary Measures Derived from a Multi-state Modeling Framework to Characterize the Course of Heart Disease. BMC Medical Research Methodology. 2026;26(1):70.
- Ding M. Estimating the Effects of Treatment Regimes over the Course of Chronic Disease: A Multi-state Causal Framework with Baseline Confounding. Statistical Methods in Medical Research. 2026

Results



Characteristics	ARIC
Number of participants (N)	13,036
Age, year	54 (49, 59)
Female, %	54 (55.2%)
Blacks, %	3,218 (24.7%)
Total cholesterol, mmol/L	212 (186, 239)
HDL cholesterol, mmol/L	49.1 (39.5, 61.6)
SBP, mmHg	130 (112, 152)
BMI, kg/m ²	26.7 (23.9, 30.2)
eGFR, mL/min per 1.73 m ²	103.2 (95, 111.4)
Diabetes, %	1,134 (8.7%)
Current Smokers, %	3,393 (26.0%)
BP medication use, %	2,837 (21.8%)
Statin use, %	60 (0.5%)
Follow-up time, year	28 (19.8, 29.2)

Table 1. Baseline Characteristics of Participants in the Ath atherosclerosis Risk in Communities (ARIC) Study. The ARICa data were obtained from the NHLBI BioLINCC under a Data Use Agreement (DUA).



	S0→S1 n=1.5k/13k	S0→S2 n=2.2k/13k	S1→S2 n=654/1.5k	S2→S1 n=410/2.2k
Age (per year)	1.05 (1.04, 1.06)	1.10 (1.09, 1.11)	1.05 (1.03, 1.06)	1.03 (1.01, 1.05)
Sex (male vs. female)	1.82 (1.62, 2.04)	1.21 (1.10, 1.33)	0.76 (0.64, 0.91)	1.12 (0.91, 1.39)
Race (black vs. non-black)	1.11 (0.98, 1.27)	1.41 (1.27, 1.56)	1.29 (1.06, 1.56)	1.46 (1.17, 1.81)
Total cholesterol (per SD)	1.29 (1.23, 1.35)	1.01 (0.97, 1.06)	0.96 (0.89, 1.03)	1.21 (1.11, 1.32)
HDL cholesterol (per SD)	0.75 (0.68, 0.82)	0.97 (0.90, 1.04)	0.95 (0.81, 1.10)	0.89 (0.75, 1.07)
SBP (per SD)	0.96 (0.89, 1.05)	1.01 (0.95, 1.09)	1.07 (0.93, 1.22)	0.98 (0.83, 1.15)
BMI (per SD)	1.07 (1.01, 1.14)	1.37 (1.31, 1.43)	1.22 (1.13, 1.33)	0.91 (0.82, 1.00)
eGFR (per SD)	0.91 (0.86, 0.97)	0.91 (0.87, 0.95)	1.00 (0.92, 1.09)	0.89 (0.83, 0.96)
Diabetes (yes vs. no)	2.23 (1.93, 2.59)	2.35 (2.09, 2.64)	1.60 (1.32, 1.95)	2.05 (1.63, 2.57)
Current smoking (yes vs. no)	1.91 (1.71, 2.14)	2.32 (2.11, 2.55)	1.25 (1.05, 1.49)	1.43 (1.16, 1.78)
BP med. use (yes vs. no)	1.42 (1.26, 1.60)	1.45 (1.32, 1.59)	1.13 (0.95, 1.35)	1.19 (0.96, 1.46)
Statin use (yes vs. no)	0.73 (0.32, 1.62)	1.01 (0.57, 1.78)	0.88 (0.22, 3.55)	1.52 (0.56, 4.11)

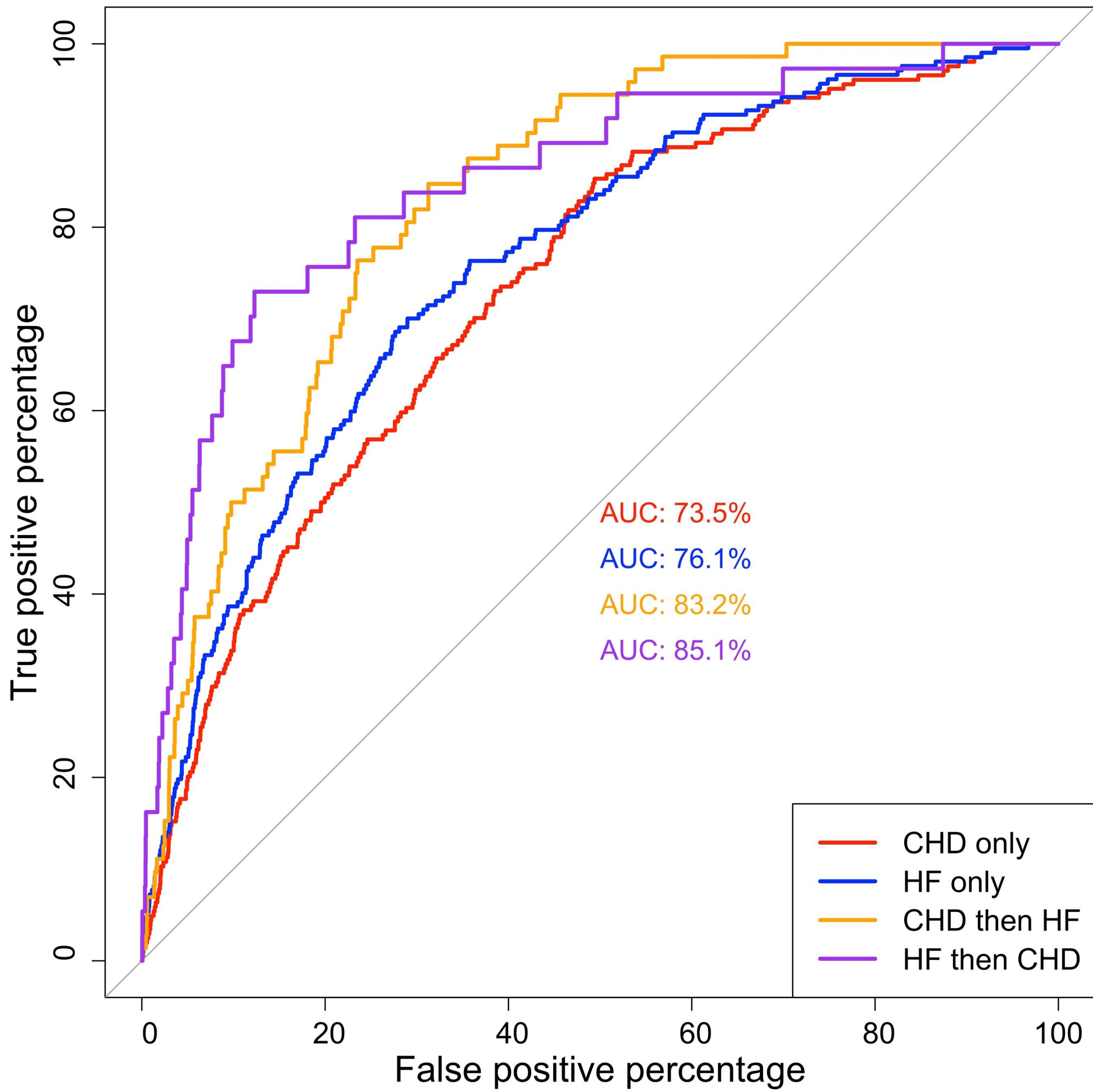
Figure 2. Multivariable-adjusted associations of baseline predictors with hazards of transition between states

Conclusion

Unlike single-endpoint tools such as PREVENT, this framework distinguishes *which* trajectory a person is likely to follow, not just *whether* an event occurs, with good discrimination and calibration.

Future Development

- External validation in independent cohorts to test generalizability across populations.
- Incorporation of time-varying predictors, updated during follow-up rather than fixed at baseline.



	CHD only	HF only	CHD—HF path	HF—CHD path
Women				
Observed cases	69	105	32	16
c-index	0.73 (0.73, 0.79)	0.77 (0.76, 0.79)	0.86 (0.84, 0.88)	0.93 (0.91, 0.95)
Predicted risk	1.91%	1.70%	0.49%	0.37%
Observed risk	1.92%	2.92%	0.89%	0.45%
Calibration slope	1.21	1.10	1.18	1.34
Men				
Observed cases	135	102	40	21
c-index	0.68(0.67, 0.70)	0.75(0.73, 0.77)	0.80(0.78, 0.82)	0.78(0.74, 0.82)
Predicted risk	4.26%	1.76%	0.81%	0.40%
Observed risk	4.69%	3.55%	1.39%	0.73%
Calibration slope	1.22	1.29	1.21	0.90

Table 2. 10-year ROC and calibration for women and men based on the CSC model including traditional risk factors.